

SARVARASA

LIFESTYLE AUDIT

Phase 2 — Product Specification for Tech Team

Prepared for:

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HOW TO READ THIS DOCUMENT

This document is split into two parts. Part 1 contains only the questions the customer sees. Part 2 contains the evaluation criteria that run silently in the backend — the customer never sees scores, labels, or flags. Build both layers independently.

PART 1 — Questions	PART 2 — Evaluation Criteria
What the customer sees on screen. One question per screen. Tap or select format. No long typing.	Backend scoring logic, interpretation guides, clinical flags. Never shown to the customer at any point.

PART 1

The Questions

What the customer sees — one screen per question. No scoring visible. No labels. No percentages. Clean, simple, respectful.

OPENING SCREEN — shown before any questions begin

"This is not a health quiz. It is a picture of your life right now. Answer based on what actually happens — not what you wish happened. There are no wrong answers. Only honest ones and polished ones. The more honest you are, the more useful your results will be."

A. BASIC PROFILE Personal Details

Collected once at account creation. Not scored — used to contextualise all other responses.

Q1 What is your full name?

Text input. Store as customer display name throughout the platform.

Q2 How old are you?

→ Number input, range 18 to 70

Q3 What is your occupation?

→ Short text input — e.g. Software Engineer, Teacher, Business Owner

Q4 What best describes your work type?

- A Desk-based — I sit at a computer for most of the day
- B Mix — some desk work, some movement
- C Active — I am on my feet or moving for most of the day
- D Shift work — my schedule changes regularly

Q5 On a typical workday, how many hours do you spend sitting?

- A** Less than 4 hours
- B** 4 to 6 hours
- C** 6 to 8 hours
- D** More than 8 hours

Q6 What is your marital status? (Optional)

- A** Single
- B** Married
- C** Other / prefer not to say

B. SLEEP AUDIT	Weight: 1.5x
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Q7 What time do you usually go to sleep on weeknights?

- A** Before 10 PM
- B** 10 PM to 11 PM
- C** 11 PM to midnight
- D** Midnight to 1 AM
- E** After 1 AM

Q8 What time do you usually wake up?

- A** Before 6 AM
- B** 6 AM to 7 AM
- C** 7 AM to 8 AM

- D After 8 AM
- E I have no fixed wake time — it depends on the day

Q9 On most nights, how many hours of actual sleep do you get?

- A 7 to 9 hours
- B 6 to 7 hours
- C 5 to 6 hours
- D Less than 5 hours
- E It varies a lot — I cannot predict it

Q10 When you wake up, how do you feel?

- A Rested and ready — I feel okay getting up
- B Groggy for 10 to 15 minutes but okay after that
- C Tired — I want to sleep more and often hit snooze
- D Heavy and exhausted — mornings are hard
- E I never feel rested, regardless of how long I sleep

Q11 What do you usually do in the last 30 minutes before sleeping?

- A I avoid screens — I read, meditate, or wind down quietly
- B I use my phone occasionally but stop before I sleep
- C I scroll social media or watch content until I feel sleepy
- D I am on my phone or a screen right until I fall asleep
- E I often fall asleep while watching something

Q12 How often do you have difficulty falling asleep?

- A Never — I fall asleep easily
- B Rarely — only occasionally
- C Sometimes — a few nights a week
- D Often — most nights
- E Almost every night

C. FOOD & EATING HABITS

Weight: 1.5x

Q13 How many proper meals do you eat on a typical weekday? (Not counting tea, coffee, or small snacks)

- A 3 or more meals at roughly consistent times
- B 2 proper meals, fairly consistent
- C 2 proper meals but timing varies a lot
- D 1 proper meal most days
- E I eat whenever I get time — no real pattern

Q14 Are your meal timings regular or irregular?

- A Very regular — roughly same time every day
- B Usually regular with occasional variation
- C Irregular — timing shifts by hours depending on work
- D Very irregular — no consistent timing at all

Q15 What time do you usually finish dinner?

- A** Before 7:30 PM most days
- B** 7:30 PM to 8:30 PM most days
- C** 8:30 PM to 9:30 PM most days
- D** After 9:30 PM most days
- E** After 10:30 PM most days

Q16 How often do you eat junk or processed food in a typical week?

- A** Rarely — once a month or less
- B** Occasionally — once or twice a week
- C** Often — 3 to 4 times a week
- D** Most days
- E** Almost every meal includes processed or outside food

Q17 How often do you eat after 9 PM or late at night?

- A** Rarely — almost never
- B** Once or twice a week
- C** 3 to 4 nights a week
- D** Most nights
- E** Every night

Q18 How would you describe your sugar cravings?

- A** Low — I rarely crave sweets or sugary food
- B** Moderate — I crave occasionally and can manage it

- C High — I frequently crave and usually give in
- D Very high — I need something sweet most days

Q19 Do you eat differently when you are stressed, bored, or upset?

- A No — my eating is not strongly linked to my mood
- B Sometimes — minor changes when stressed
- C Often — I eat more or reach for specific foods when stressed
- D Yes — food is my primary way of managing difficult emotions

Q20 How much water do you drink in a typical day?

- A 2 litres or more
- B 1 to 2 litres
- C Less than 1 litre
- D I am not sure — I do not track it

D. MOVEMENT & ACTIVITY	Weight: 1x
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Q21 How many days per week do you do any intentional physical activity? (Walking, exercise, yoga, sports — anything purposeful)

- A 5 or more days
- B 3 to 4 days
- C 1 to 2 days
- D Less than once a week
- E I do not do any intentional physical activity

Q22 On a typical day, how much do you walk in total?

- A** More than 30 minutes of walking
- B** 15 to 30 minutes
- C** Less than 15 minutes
- D** Minimal — I take vehicles or lifts for almost everything

Q23 How long do you sit continuously without getting up on a typical workday?

- A** Rarely more than 1 hour without moving
- B** 1 to 2 hours at a stretch sometimes
- C** 2 to 3 hours regularly without a break
- D** 3 to 4 hours without getting up
- E** 4 or more hours without getting up

Q24 Do you experience any regular body pain or stiffness?

- A** No — my body feels fine generally
- B** Occasional minor stiffness — usually in the morning
- C** Regular lower back or neck pain from sitting
- D** Daily pain or stiffness that affects how I move

Q25 How would you describe your energy levels during the day?

- A** Consistently good — no significant dips
- B** Good in the morning, slight dip in the afternoon
- C** Noticeable afternoon crash — I need tea or coffee to recover

- D Low energy most of the day
- E Exhausted for most of the day regardless of sleep

E. STRESS & EMOTIONAL HEALTH

Weight: 1x

Q26 On a scale of 1 to 10, how would you rate your current stress level? (1 = very low, 10 = overwhelming)

→ Scale selector — 1 to 10

Q27 What is your biggest source of stress right now?

- A Work pressure — deadlines, performance, or relationships at work
- B Family or relationship situations
- C Financial concerns
- D Health concerns — my own or someone close to me
- E Multiple areas simultaneously
- F I am not under significant stress right now

Q28 How often do your mood or emotional state fluctuate significantly during the day?

- A Rarely — I am generally emotionally steady
- B Occasionally — minor fluctuations
- C Often — I notice significant shifts in mood most days
- D Very often — my mood is unpredictable and hard to manage

Q29 What is your primary motivation for joining this program?

- A A specific health concern or doctor's advice
- B I want to lose weight and feel better in my body
- C I want more energy and better daily function
- D I want to build sustainable habits before problems start
- E Someone I know recommended it

Q30 What has been your biggest challenge in maintaining healthy habits in the past?

- A I do not have enough time
- B I start but cannot stay consistent
- C I lack support or accountability
- D I find healthy food difficult to access or prepare
- E Stress and emotional state derail me
- F I have not seriously tried before

F. DIGESTIVE & FUNCTIONAL HEALTH	Weight: 1.5x
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Q31 How often do you experience bloating or a feeling of fullness and discomfort after meals?

- A Rarely — only when I eat something unusual
- B Occasionally — once or twice a week
- C Often — several times a week
- D Most days
- E Almost every meal

Q32 How would you describe your bowel movement pattern?

- A** Regular — once a day at roughly the same time
- B** Mostly regular — once a day but timing varies
- C** Once every 2 days or less frequently
- D** Irregular — I cannot predict when
- E** Constipation is a persistent concern for me

Q33 How often do you experience acidity, heartburn, or a burning sensation in your chest or stomach?

- A** Never or very rarely
- B** Once or twice a month
- C** Once or twice a week
- D** Most days
- E** Multiple times per day

Q34 How would you describe your appetite?

- A** Consistent — I feel hungry at regular times and stop when full
- B** Slightly variable — some days more hungry than others
- C** Unpredictable — my hunger fluctuates significantly
- D** I rarely feel genuinely hungry — I eat by habit or schedule
- E** I frequently overeat past the point of fullness

Q35 How do you feel in the 30 to 60 minutes after a typical meal?

- A** Light and energised — I feel good after eating

- B** Neutral — neither energised nor heavy
- C** A little heavy or sluggish but it passes
- D** Heavy and tired — I often want to lie down or rest
- E** Very heavy and uncomfortable — this happens almost every meal

PART 2

Evaluation Criteria

Backend scoring logic, interpretation guides, and clinical flags. The customer never sees any of this. These criteria run silently in the scoring engine and populate the clinical team's dashboard.

SCORING ARCHITECTURE

Domain	Weight	Max Contribution	Why This Weight
A — Basic Profile	Not scored	0 pts	Context data only — informs interpretation
B — Sleep	1.5x	15 pts	Sleep deprivation directly impairs insulin sensitivity and elevates cortisol
C — Food & Eating	1.5x	15 pts	Primary metabolic driver — food timing and quality are the core interventions
D — Movement	1x	10 pts	Important but secondary to food in early metabolic intervention
E — Stress	1x	10 pts	Addressed through habit stacking rather than direct early intervention
F — Digestion	1.5x	15 pts	Direct observable signal of metabolic function — early MetS marker

TOTAL		65 pts	
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B. SLEEP AUDIT — Scoring Criteria	Max 15 pts
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Q7 — Sleep time

Score	Label	What It Means	Action
A	Before 10 PM	Optimal circadian alignment	No flag
B	10–11 PM	Acceptable — minor risk	No flag
C	11 PM–midnight	Moderate risk — cortisol disruption possible	Yellow flag
D	Midnight–1 AM	High risk — chronic sleep phase delay	Orange flag
E	After 1 AM	Critical — metabolic and cognitive impairment likely	Red flag

Q8 — Wake time

Score	Label	What It Means	Action
A	Before 6 AM	Excellent — early cortisol peak utilised	No flag
B	6–7 AM	Good — healthy circadian pattern	No flag
C	7–8 AM	Acceptable	No flag
D	After 8 AM	Late — check against sleep time for total hours	Yellow flag

E	No fixed time	Circadian disruption — inconsistency is itself a risk	Orange flag
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Q9 — Sleep duration

Score	Label	What It Means	Action
A	7–9 hrs	Optimal — full sleep architecture completed	No flag
B	6–7 hrs	Slightly below — minor risk over time	No flag
C	5–6 hrs	Sleep deprivation — metabolic risk elevated	Yellow flag
D	<5 hrs	Severe — insulin resistance and cortisol elevation likely	Red flag
E	Variable	Inconsistency compounds risk regardless of average	Orange flag

Q10 — Morning feeling (most clinically significant sleep question)

Score	Label	What It Means	Action
A	Rested	Sleep is restorative — quality is good	No flag
B	Groggy briefly	Minor sleep inertia — acceptable	No flag
C	Tired	Non-restorative sleep despite adequate hours — investigate	Yellow flag
D	Heavy and exhausted	Significant sleep dysfunction — check Q9 hours	Orange flag
E	Never rested	May indicate sleep disorder — clinical review required	Red flag

■ **Clinical note:** Q10 Option E combined with Q9 Option A or B (sufficient hours but no rest) is a clinical flag for potential sleep apnoea or other sleep disorder. Flag for BNYS review.

Q11 — Screen use before sleep

Score	Label	What It Means	Action
A	No screens	Optimal melatonin production — ideal	No flag
B	Occasional	Minor impact — acceptable	No flag
C	Scrolls until sleepy	Blue light suppression of melatonin — primary intervention point	Yellow flag
D	Screen until sleep	Significant melatonin disruption — links to Q9 and Q10	Orange flag
E	Falls asleep to screens	Habitual — hard to break, affects sleep architecture	Orange flag

Q12 — Difficulty falling asleep

Score	Label	What It Means	Action
A	Never	No sleep onset concern	No flag
B	Rarely	Minor — within normal range	No flag
C	Sometimes	Moderate — check stress score (Q26)	Yellow flag
D	Often	Chronic onset insomnia — flag for review	Orange flag
E	Almost every night	Clinical sleep disorder possible — mandatory review	Red flag

$B_raw = \text{average of } (Q7 + Q8 + Q9 + Q10 + Q11 + Q12) \text{ scores} / 6$
 $B_weighted = B_raw \times 1.5$ | Maximum: 15 pts

Domain B — Interpretation Guide for Clinical Team

B Raw Score	Level	Clinical Meaning	First Intervention
8–10	Strong	Sleep is not the primary concern	No sleep-specific action needed in Week 1
5–7	Moderate	Sleep timing or screen use affecting quality	Introduce early dinner protocol — this alone improves sleep within 2 weeks
2–4	Significant	Sleep dysfunction — cortisol elevated, metabolic risk active	Sleep timing anchor first — fixed wake time before any food changes
0–1	Critical	Severe deprivation — cannot start other interventions effectively	BNYS review before program start. Referral if unexplained.

C. FOOD & EATING HABITS — Scoring Criteria

Max 15 pts

Q13 — Meal regularity

Score	Label	What It Means	Action
A	3+ regular meals	Strong foundation — reinforce meal structure	No flag
B	2 regular	Acceptable if timing is consistent	No flag
C	2 irregular	Timing variability is itself a metabolic risk	Yellow flag
D	1 meal	High risk — compensatory eating pattern likely	Orange flag

E	No pattern	No structure — highest metabolic risk from meal timing	Red flag
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Q14 — Meal timing consistency

Score	Label	What It Means	Action
A	Very regular	Optimal circadian feeding alignment	No flag
B	Usually regular	Minor variation — acceptable	No flag
C	Irregular	Circadian disruption — increases insulin variability	Yellow flag
D	Very irregular	No circadian anchor — clinical priority to establish one	Orange flag

Q15 — Dinner timing (highest clinical significance in domain C)

Score	Label	What It Means	Action
A	Before 7:30 PM	Optimal — adequate overnight fast, sleep quality protected	No flag
B	7:30–8:30 PM	Acceptable — minor risk	No flag
C	8:30–9:30 PM	Late eating beginning to impair overnight metabolic rest	Yellow flag
D	After 9:30 PM	High risk — insulin spike during melatonin production window	Orange flag
E	After 10:30 PM	Critical — direct MetS risk factor, triglyceride accumulation likely	Red flag

■ **Clinical note:** Q15 option D or E for pre-diabetic customers is the single highest priority intervention in the entire program. Early dinner must be the first habit introduced regardless of other scores.

Q16 — Junk or processed food frequency

Score	Label	What It Means	Action
A	Rarely	Good — outside food is occasional	No flag
B	1–2 times/week	Acceptable — manageable through food order protocol	No flag
C	3–4 times/week	Moderate risk — refined carbs likely dominant	Yellow flag
D	Most days	High processed food load — gut and glucose impact	Orange flag
E	Almost every meal	Critical — food quality overhaul needed before any other change	Red flag

Q17 — Late night eating

Score	Label	What It Means	Action
A	Rarely	No late eating concern	No flag
B	1–2 nights/week	Minor — not a primary concern	No flag
C	3–4 nights/week	Moderate — disrupting overnight metabolic rest	Yellow flag

D	Most nights	High — directly linked to abdominal fat accumulation	Orange flag
E	Every night	Critical — combined with Q15 this is the priority intervention pair	Red flag

Q18 — Sugar cravings

Score	Label	What It Means	Action
A	Low	No significant sugar-dependency pattern	No flag
B	Moderate, managed	Acceptable — food order will help reduce this	No flag
C	High, gives in	Likely linked to glucose spikes from poor food order	Yellow flag
D	Very high, daily	Possible insulin resistance pattern — flag for Q6.4 check	Orange flag

Q19 — Emotional eating

Score	Label	What It Means	Action
A	Not linked to mood	Food relationship is healthy	No flag
B	Minor stress link	Common — manageable through habit stacking	No flag
C	Often stress-driven	Emotional eating pattern — note for SRA dimension 2	Yellow flag
D	Primary coping	Clinical concern — flag for psychology review in SRA phase	Orange flag

■ **Clinical note:** Q19 option D combined with Q26 stress score above 7 is a dual-flag combination. Refer for resilience coaching layer before program start.

Q20 — Water intake

Score	Label	What It Means	Action
A	2L+	Optimal hydration	No flag
B	1–2L	Acceptable — morning water habit will close the gap	No flag
C	<1L	Under-hydrated — morning water protocol is first habit	Yellow flag
D	Not tracked	No awareness — introduce water tracking in Week 1	Yellow flag

$C_{raw} = \text{average of } (Q13+Q14+Q15+Q16+Q17+Q18+Q19+Q20) \text{ scores} / 8$
 $C_{weighted} = C_{raw} \times 1.5 \quad | \quad \text{Maximum: 15 pts}$

Domain C — Interpretation Guide

C Raw Score	Level	Clinical Meaning	First Intervention
8–10	Strong	Good food foundation — food order optimisation is the lever	Introduce food sequence protocol in Week 2
5–7	Moderate	Timing and source issues — significant improvement possible	Early dinner first. Food order second. Food source third.

2–4	Significant	Multiple food systems compromised simultaneously	Dinner timing as the single first intervention. One change only in Week 1.
0–1	Critical	Food habits are the primary driver of all MetS markers	BNYS review mandatory. Lab panel required. Do not start program without clinical sign-off.

D. MOVEMENT & ACTIVITY — Scoring Criteria **Max 10 pts**

Q21 — Exercise frequency

Score	Label	What It Means	Action
A	5+ days/week	Active lifestyle — movement is not the primary lever	No flag
B	3–4 days	Good movement habit present	No flag
C	1–2 days	Minimal intentional movement — Surya Namaskar protocol relevant	Yellow flag
D	<Once/week	Near-sedentary — begin with 2-minute movement breaks	Orange flag
E	None	Completely sedentary — start only with walking before any structured exercise	Orange flag

Q22 — Daily walking

Score	Label	What It Means	Action
A	30+ min	Good baseline activity	No flag

B	15–30 min	Acceptable — improve gradually	No flag
C	<15 min	Low incidental movement — suggest 10-min post-meal walk	Yellow flag
D	Minimal	Avoidance pattern — vehicles and lifts for everything	Orange flag

Q23 — Sitting duration (sedentary risk)

Score	Label	What It Means	Action
A	<1 hr stretches	Good movement pattern within work	No flag
B	1–2 hrs	Minor risk — standing breaks sufficient	No flag
C	2–3 hrs	Moderate sedentary risk — hourly break protocol	Yellow flag
D	3–4 hrs	High sedentary risk — musculoskeletal and metabolic	Orange flag
E	4+ hrs	Critical sedentary pattern — links to all metabolic markers	Red flag

Q24 — Body pain or stiffness

Score	Label	What It Means	Action
A	None	Body feels fine — no movement limitation	No flag
B	Minor morning stiffness	Common — movement protocol will address	No flag

C	Regular back or neck pain	Likely from Q23 high sitting — posture and breaks first	Yellow flag
D	Daily pain affects movement	Clinical review before prescribing movement protocol	Orange flag

Q25 — Daytime energy (also cross-references with domain C and F)

Score	Label	What It Means	Action
A	Consistently good	Energy is not a primary complaint	No flag
B	Slight afternoon dip	Very common — food order change will resolve within 5–7 days	No flag
C	Afternoon crash	Post-prandial glucose spike — high priority food order flag	Yellow flag
D	Low most of day	Multiple factors — check sleep (B), food timing (C), digestion (F)	Orange flag
E	Exhausted all day	Burnout or clinical deficiency — flag for lab panel	Red flag

■ **Clinical note:** Q25 option C is a primary indicator of post-prandial glucose spike. Food order protocol is the direct intervention. Customers scoring C on Q25 should be told within the first week: 'The afternoon crash you experience is likely caused by how you sequence your food, not what you eat.'

$D_{raw} = \text{average of } (Q21+Q22+Q23+Q24+Q25) \text{ scores} / 5$
 $D_{weighted} = D_{raw} \times 1.0$ | Maximum: 10 pts

E. STRESS & EMOTIONAL HEALTH — Scoring Criteria **Max 10 pts**

Q26 — Stress level scale 1–10 (convert to 0–10 scoring)

Scale Value	Score Given	Interpretation	Action
1–3	10	Low stress — not a primary concern	No flag
4–5	7	Moderate — manageable with current coping	Monitor — check Q28 and Q30
6–7	4	Elevated — stress is affecting daily function	Yellow flag — Week 4 re-engagement risk
8–9	2	High chronic stress — cortisol actively elevating metabolic risk	Orange flag — resilience coaching layer recommended
10	0	Overwhelming — programme adherence severely at risk	Red flag — discuss readiness in SRA before admitting

Q27 — Primary stress source (context — not directly scored)

Store verbatim. Use to personalise Week 4 re-engagement message. Cross-reference with Q30. Multiple simultaneous sources (option E) increases dropout risk — flag in Protocol Card.

Q28 — Mood fluctuations

Score	Label	What It Means	Action
A	Steady	Emotional regulation healthy	No flag
B	Minor	Within normal variation	No flag
C	Often fluctuates	May be linked to blood sugar variability — cross check Q25	Yellow flag

D	Very unpredictable	Clinical concern — flag for psychology input in SRA phase	Orange flag
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■ **Clinical note:** Q28 option C combined with Q25 option C (afternoon energy crash) is a strong indicator of post-prandial glucose dysregulation affecting mood. Food order protocol addresses both simultaneously.

Q29 — Motivation for joining (context — informs SRA framing)

Store verbatim. Not directly scored. Feeds into SRA Dimension 1 context. Option A (health concern or doctor's advice) paired with high D6 score (body signals) indicates genuine urgency — prioritise for current cohort.

Q30 — Biggest past challenge (feeds directly into SRA Dimension 2)

Opt	Challenge	Mental Model Indicator	SRA Flag
A	No time	The Disruptor — habit attached to schedule not identity	Pre-populate SRA Q6 probe question
B	Cannot stay consistent	The Self-Saboteur or Complexity Collapse	Pre-populate SRA Q6 with consistency-specific probe
C	No accountability	The Isolated Striver — needs peer challenge mechanic	Flag for accountability group placement
D	Food access	Practical barrier — food module discussion needed	Flag for food module recommendation
E	Stress derails	The Escapist — emotional eating confirmed	Cross-check Q19. Resilience coaching layer recommended.
F	Never tried	First attempt — no prior track record	Apply first-attempt pathway from SRA spec

$E_raw = \text{average of (Q26_converted + Q28) scores} / 2$
 $E_weighted = E_raw \times 1.0$ | Maximum: 10 pts (Q27, Q29, Q30 are context fields — stored, not scored)

F. DIGESTIVE & FUNCTIONAL HEALTH — Scoring Criteria

Max 15 pts

Q31 — Bloating frequency

Score	Label	What It Means	Action
A	Rarely	Gut function is healthy — food order will maintain this	No flag
B	1–2×/week	Minor — food order protocol will reduce within 2 weeks	No flag
C	Several times/week	Moderate gut dysfunction — food sequence is primary fix	Yellow flag
D	Most days	Significant — chewing protocol and food order both required	Orange flag
E	Every meal	Critical — investigate food source and speed of eating first	Red flag

Q32 — Bowel regularity

Score	Label	What It Means	Action
A	Daily, regular	Optimal gut motility	No flag
B	Daily, variable	Acceptable — minor variation	No flag

C	Every 2+ days	Moderate constipation — hydration and fibre first	Yellow flag
D	Irregular	Gut motility disrupted — check Q20 water and Q31 bloating	Orange flag
E	Persistent constipation	Clinical — may need BNYS dietary protocol specific to this	Orange flag

Q33 — Acidity or heartburn

Score	Label	What It Means	Action
A	Never	No acid reflux concern	No flag
B	1–2×/month	Occasional — late eating likely cause	No flag
C	1–2×/week	Moderate — check Q15 dinner timing — strong correlation	Yellow flag
D	Most days	Chronic — early dinner protocol is primary intervention	Orange flag
E	Multiple times/day	Clinical — may require medical review alongside program	Red flag

■ **Clinical note:** Q33 options D or E must be flagged for BNYS review. Chronic acidity can be symptom of GERD, H. pylori, or peptic ulcer — all require medical management alongside lifestyle changes.

Q34 — Appetite pattern

Score	Label	What It Means	Action
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A	Consistent, stops when full	Healthy hunger-satiety signalling	No flag
B	Slightly variable	Normal variation — not a concern	No flag
C	Unpredictable	May be linked to glucose dysregulation — check Q25	Yellow flag
D	Rarely genuinely hungry	May signal leptin resistance — flag with Q25 and Q18	Orange flag
E	Frequently overeats	Satiety signalling disrupted — chewing protocol is primary fix	Orange flag

Q35 — Post-meal energy (highest clinical significance in domain F)

Score	Label	What It Means	Action
A	Light and energised	Excellent post-prandial response — food order likely good	No flag
B	Neutral	Acceptable — food order will improve this	No flag
C	Heavy briefly	Mild post-prandial fatigue — food order intervention	Yellow flag
D	Heavy and tired, wants to rest	Significant glucose spike — food order is the primary clinical intervention	Orange flag
E	Heavy and uncomfortable every meal	Critical — combined with Q31 this indicates severe gut-metabolic dysfunction	Red flag

■ **Clinical note:** Q35 option D or E combined with Q31 option D or E is a dual-flag combination. This pattern indicates simultaneous glucose dysregulation and gut dysfunction. Food order protocol should be started in Week 1 and results measured at Week 2 check-in.

$F_raw = \text{average of } (Q31+Q32+Q33+Q34+Q35) \text{ scores} / 5$
 $F_weighted = F_raw \times 1.5$ | Maximum: 15 pts

Domain F — Interpretation Guide

F Raw Score	Level	Clinical Meaning	First Intervention
8–10	Good gut function	Food order will maintain and improve	Introduce food order as an enhancement — not a fix
5–7	Moderate dysfunction	Multiple gut signals present	Food order and chewing — visible improvement within 10–14 days. Use as first win.
2–4	Significant	Likely caused by late eating, poor chewing, high outside food	BNYS clinical diet review before program starts
0–1	Severe	Gut dysfunction is affecting energy, mood, and metabolic function	BNYS mandatory. Possible referral needed.

TOTAL SCORE CALCULATION & ZONE CLASSIFICATION

$TOTAL\ SCORE = B_weighted + C_weighted + D_weighted + E_weighted + F_weighted$
 $= (B \times 1.5) + (C \times 1.5) + (D \times 1.0) + (E \times 1.0) + (F \times 1.5)$
 Maximum possible score = $15 + 15 + 10 + 10 + 15 = 65$ pts
 Domain A (Basic Profile) — not included in score
 Q27, Q29, Q30 — context fields only, not included in score

The Four Zones

Zone	Score Range	What It Means	Clinical Team Action
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 GREEN	50–65	Strong lifestyle foundation. Minor optimisation. Not in early MetS risk territory.	Standard onboarding. Program starts at Week 2 habits — foundational reset not needed.
 YELLOW	38–49	Habits drifting. Gradual lifestyle erosion. Customer can feel the difference even if they have not named it.	Flag lowest-scoring domain as Week 1 priority. Standard onboarding.
 ORANGE	25–37	Core Sarvarasa customer. Multiple systems compromised. Early-to-moderate MetS risk. Feels the effects daily.	BNYS review at onboarding call mandatory. Baseline lab panel required before Week 1.
 RED	0–24	Significant lifestyle dysfunction. Clear MetS markers. Clinical support urgently needed.	BNYS consultation mandatory before program design. Labs required urgently. Do not start without clinical review.

Customer-Facing Zone Messaging

What to tell the customer — these messages appear in the app after score calculation. Never show the score number — show the zone name and message only.

Zone	Message Shown to Customer
	<i>"Your lifestyle is working for you in most areas. The program will sharpen what is already good and build on a strong foundation. You are in the right place to get ahead of problems rather than reverse them."</i>
	<i>"Your lifestyle has been drifting — not dramatically, but consistently. The good news is that drifting is reversible faster than it accumulated. The program will give you the structure to reverse it before it becomes a more serious concern."</i>
	<i>"Your score tells us that several areas of your lifestyle are working against your health at the same time. This is the most common pattern we see — and also the pattern that responds most dramatically to structured intervention. You are in exactly the right place."</i>



"Your results show that your body has been under sustained pressure for some time. We are glad you are here. Before we design your program, one of our clinical team members will reach out to understand your situation personally."

CRITICAL FLAG COMBINATIONS — Auto-escalate to BNYS review

When any of these combinations occur, the system must auto-flag the profile as BNYS Review Required = true, regardless of total zone score.

Combination	Clinical Signal	Action
Q15 D/E + Q17 D/E	Late dinner AND late night eating — highest MetS risk pairing	Red flag. BNYS mandatory. Labs required.
Q10 E + Q9 A/B	Never rested despite sufficient sleep hours — possible sleep disorder	Orange flag. BNYS review. Consider sleep study referral.
Q35 D/E + Q31 D/E	Heavy post-meal + frequent bloating — significant gut-metabolic dysfunction	Orange flag. BNYS diet review before Week 1.
Q33 D/E	Daily or multiple-daily acidity — possible GERD or peptic condition	Orange/Red flag. Medical review may be required.
Q26 score 8–10 + Q19 D	Overwhelming stress + emotional eating as primary coping — dropout risk very high	Flag for resilience coaching layer. Discuss SRA before admitting.
Q28 D + Q25 C/D	Mood fluctuations + afternoon crash — glucose dysregulation affecting mood	Food order protocol is the direct intervention for both simultaneously.

DATA STRUCTURE — Fields to store per customer

```
lifestyle_audit: { section_a: { name, age, occupation, work_type, sitting_hours,
marital_status }, section_b: { q7-q12 scores, b_raw, b_weighted, sleep_flags[] }, section_c:
{ q13-q20 scores, c_raw, c_weighted, food_flags[] }, section_d: { q21-q25 scores, d_raw,
d_weighted, movement_flags[] }, section_e: { q26-q30 scores/text, e_raw, e_weighted,
stress_flags[] },      q27_text (verbatim), q29_text (verbatim), q30_selection, section_f:
{ q31-q35 scores, f_raw, f_weighted, digestion_flags[] }, total_score: float, zone: enum
[Green, Yellow, Orange, Red], lowest_domain: string, highest_domain: string, critical_flags:
[], bnys_review_required: boolean, baseline_labs_required: boolean, priority_intervention:
string, mental_model_indicator: string // from Q30 mapping}
```

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